SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Nivolumab every 2 weeks for head and neck cancer

Recurrent or metastatic squamous cell carcinoma of the head and neck after platinum treatment; the cancer cannot be treated with curative local therapy and has progressed or returned within six months of the last platinum dose. · NCCP 00483 · version 13a · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Nivolumab aims to control head and neck cancer that has returned or spread, relieve symptoms and help some people live longer. Benefit and side effects are reviewed for you.

Schedule
Nivolumab is given as 240 mg through a vein over 30 minutes OR 600 mg as an injection under the skin over 3–5 minutes, on Day 1 every 2 weeks. Your team selects one formulation; these doses are not interchangeable milligram for milligram. Allow extra time for safety checks, preparation and observation. Treatment continues until the cancer progresses or side effects become unacceptable, with individual benefit and safety review. There is no fixed number of cycles in this head and neck treatment course.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart and nervous system
- ☐ Infusion or injection reactions
- ☐ Some effects need steroids, hospital care or long-term hormone replacement
- ☐ Effects may start after the final dose

Discuss your diagnosis, intent, alternatives, expected benefit and uncertainty, treatment burden, serious or lasting harms, fertility and pregnancy precautions and your priorities. Include immune inflammation of lungs, bowel, liver, kidneys, skin, glands, heart, muscles, nerves and eyes; infusion or injection reactions; delayed effects; possible high-dose steroid treatment, permanent hormone replacement or permanent treatment stopping. Confirm actual urgent contacts and use teach-back. This draft does not authorise treatment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00483 v13a.

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Developed by Dr Paul O'Brien